INFORMATION TO KEEP WITH YOU

My complete treatment handbook

Weekly paclitaxel · relapsed or refractory small-cell lung cancer (SCLC)

Understand your treatment. Know what to expect. Keep your plan to hand.

Displayed calendar
28 days
Intervals differ by treatment phase

Treatment stages
2
Explained in the treatment reference

Regimen source
NCCP 00226
Version 9

Paclitaxel 80 mg/m² is infused on Days 1, 8, 15 and 22 of a 28-day cycle. The consultant may instead prescribe Days 1, 8 and 15. Each planned weekly dose needs review. Continue while helping and tolerated; NCCP does not set a fixed cycle cap.

Treatment stages

01 **Days 1, 8, 15 and 22**
Four weekly visits

02 **Days 1, 8 and 15 (if prescribed)**
Three visits + an interval

Your team confirms the active stage and dates. The full treatment guide explains each step.

Inside your handbook

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My name _____

Hospital / oncology team _____

Use the urgent instructions and contact numbers in your treatment guide. Your team must replace example numbers before patient use. Your individual treatment plan takes precedence.

CHAPTER 01

Understanding your treatment

IN THIS CHAPTER

Why this treatment is offered, what its medicines do, and how the agreed plan fits your care.

When to get help

Emergency: call 112 / 999

Severe breathing difficulty, collapse, seizure or severe chest pain: call emergency services. Do not wait for an online answer.

Contact your oncology service immediately

Temperature 37.5°C or higher, 36°C or lower, shivering or suddenly feeling unwell — even with a normal temperature. Call the oncology contact number immediately for fever, shivers or sudden illness, severe or new breathing difficulty, chest pain, fainting, severe abdominal pain or persistent diarrhoea. Tell staff immediately about infusion symptoms. Report new numbness, weakness, painful skin changes, swelling or visual changes promptly; do not wait for the next cycle.

Follow the urgent contact instructions from your treating hospital. If you cannot reach the team, seek urgent medical assessment. Outside clinic hours, use your instructed out-of-hours pathway or the emergency department.

This page does not diagnose symptoms or decide whether it is safe to stay at home. You do not need to know which medicine caused a symptom before calling.

Your medicines and what they do

Treatment aims to control small-cell lung cancer after previous therapy and may help symptoms. The evidence is limited, so discuss likely benefit, burdens and alternatives with the team.

TAXANE CHEMOTHERAPY

Paclitaxel

Taxol (conventional paclitaxel)



Locks the cell division machinery

Cells use tiny structures called microtubules to separate their chromosomes when dividing. Paclitaxel stabilises these structures so they cannot rearrange normally, interrupting cell division. It can also affect nerves and other healthy tissues.

Microtubules form → Structures remain locked → Division is interrupted

What to watch for

Report tingling or numbness before the next weekly dose, especially if it affects walking or hand use. Tell the nurse immediately about flushing, chest or throat tightness, itching, dizziness or breathing changes during infusion.

The drawings are simplified explanations of drug action, not anatomical pictures or a prediction of response.

Your lung treatment plan

Exact cancer setting

NCCP 00226d: small-cell lung cancer that has returned or has not responded to previous treatment. This use is outside the Irish licence and requires the hospital off-label approval and consent process. This guide is for conventional paclitaxel alone.

Course and visits

Paclitaxel 80 mg/m² is infused on Days 1, 8, 15 and 22 of a 28-day cycle. The consultant may instead prescribe Days 1, 8 and 15. Each planned weekly dose needs review. Continue while helping and tolerated; NCCP does not set a fixed cycle cap.

What the evidence can tell us

A small uncontrolled study found some tumour responses to weekly paclitaxel in previously treated SCLC. The study used six weekly doses followed by two weeks without treatment, which differs from this NCCP schedule. It cannot establish a survival advantage over other treatment or supportive care.

Why this treatment may be chosen

Discuss possible benefit, alternatives and uncertainty for your own situation. The treatment aim is cancer control and symptom relief.

CHAPTER 02

Your schedule, tests and appointments

IN THIS CHAPTER

Your treatment stages, checks and practical preparations, from before treatment through follow-up.

Treatment pattern

Treatment stages

01 **Days 1, 8, 15 and 22**
Four weekly visits

02 **Days 1, 8 and 15 (if prescribed)**
Three visits + an interval

Your team confirms the active stage and dates. The full treatment guide explains each step.

This shows treatment timing, not guaranteed symptom-free days. Your team confirms the active phase and dates.

FOUR WEEKLY VISITS

Days 1, 8, 15 and 22

D 1 | D 8 | D 15 | D 22

Four weekly infusion visits within each 28-day cycle. The team reviews safety before each planned dose.

THREE VISITS + AN INTERVAL

Days 1, 8 and 15 (if prescribed)

D 1 | D 8 | D 15 | Interval

Day 22 is omitted only if the three-visit schedule is prescribed. Follow your confirmed appointments and prescription.

The team confirms your actual prescription and appointments. This map explains the pattern and does not decide whether a dose can be given.

BASELINE → TREATMENT → FOLLOW-UP

When checks happen

Relative stages explain the order of care; they are not equally spaced appointments. full blood count (FBC), kidney and liver profile at baseline and before each treatment; NCCP also explicitly lists Day 8 FBC. Check neuropathy, daily function, infection and previous infusion reactions before every dose. Confirm whether Day 22 is prescribed.

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00226 v9 · official regimen · HPRA conventional paclitaxel SmPC; 9 January 2026

THE WHOLE TREATMENT JOURNEY

Weekly paclitaxel · relapsed or refractory SCLC: treatment and checks

Lung · relapsed / refractory SCLC

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	Early treatment	Ongoing treatment	Pause / dose change	Completion / next decision
Treatment plan NCCP 00226	Confirm indication and prescription	Days 1/8/15/22 every 28 days	Confirm Day 22 or consultant-selected omission	Clinician review before restart	Agree the next plan
Blood / kidney / liver NCCP 00226 · SMPC	Baseline FBC, kidney and liver tests	Before each weekly dose	Repeat before each planned dose	Check recovery before restarting	Follow unresolved abnormalities
Symptoms / premedication NCCP 00226 · SMPC	Review symptoms, medicines and reaction history	Check premedication, neuropathy and function	Review before every treatment	Record worst and current symptoms; clinician confirms changes	Review persistent symptoms and recovery
Heart / breathing NCCP 00226 · SMPC	Assess history and symptoms	Infusion observations and symptom review	Extra tests if symptoms or risk require	Urgent assessment for chest pain, fainting or new breathlessness	Agree follow-up for unresolved symptoms
Cancer / course NCCP 00226 · SMPC	Confirm 00226d SCLC and off-label consent	Agree response review and actual appointment plan	Continue while beneficial and tolerated	Reassess benefit, toxicity and priorities	Agree next treatment or supportive-care plan

What each check is for

1

Safety review

Before each weekly treatment

FBC, kidney and liver profile at baseline and before each treatment; NCCP also explicitly lists Day 8 FBC. Check neuropathy, daily function, infection and previous infusion reactions before every dose. Confirm whether Day 22 is prescribed.

Ask / act: The team reviews results, current symptoms and the prescription.

2

Benefit and treatment choice

At consultant-agreed response reviews

A small uncontrolled study found some tumour responses to weekly paclitaxel in previously treated SCLC. The study used six weekly doses followed by two weeks without treatment, which differs from this NCCP schedule. It cannot establish a survival advantage over other treatment or supportive care.

Ask / **act:** Discuss continuing, changing or stopping treatment according to benefit, toxicity and priorities.

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Your practical treatment plan

STEP 1

Before the visit

Check the next weekly appointment and whether Day 22 is included. Blood results and symptom review are needed before every dose.

STEP 2

At the unit

Paclitaxel is given through a cannula or suitable existing intravenous (IV) line over about one hour. The appointment also includes checks, preparation, premedication and observation and is longer than the infusion itself. This regimen has no routine take-home pump.

STEP 3

At home

Keep the contact number, medicine list and symptom record available. Ask who will arrange your next blood test, scan and review. Sedating medicines may affect driving.

Checks through treatment

Your team arranges tests, reviews results and confirms each dose.

Blood tests and symptoms

Heart and breathing

Check observations during infusion, especially early in the infusion. Significant conduction problems require a specialist cardiac-monitoring plan. Routine serial echocardiograms are not imposed for this monotherapy.

Response review

The consultant agrees when scans and other disease checks are due. New or worsening symptoms may need assessment sooner. There is no automatic fixed scan interval in this regimen.

CHAPTER 03

Symptoms and supportive care

IN THIS CHAPTER

Recognising changes, knowing when to call, and using your prescribed supportive-care plan.

Days 1, 8, 15 and 22

Day 1 → Day 28: a general pattern

This is a visual orientation, not a prediction of your personal cycle. It shows when some effects often emerge and which effects are **not** tied to a particular day.

Four weekly infusion visits within each 28-day cycle. The team reviews safety before each planned dose.

Treatment or check	Timing in this view
Paclitaxel Day 1	 **Day 1** intravenous (IV)
Paclitaxel Day 8	 **Day 8** IV
Paclitaxel Day 15	 **Day 15** IV
Paclitaxel Day 22 · if prescribed	 **Day 22** IV
Nerves / muscle aches	 **Days 1–28** May persist or accumulate
Blood counts / energy	 **Days 1–28** Review before each dose; report illness any day

Weekly visits: Each planned infusion needs its own symptom and blood-test review.

Day 22: The fourth visit applies only to the four-visit plan. Some patients are prescribed Days 1, 8 and 15; follow your confirmed schedule.

Nerves and aches: Aches can follow treatment; numbness, tingling and fatigue may persist or accumulate. Describe changes before the next dose.

Blood counts: A gap between visits does not exclude infection or another treatment complication. Fever or sudden illness needs immediate contact.

Why these bands stay broad: Bars show planned treatment and possible symptom patterns, not measured risk. Side effects can overlap and persist. Band length does not show likelihood or severity, and the chart cannot tell you whether it is safe to receive treatment. NCCP 00226 v9 · official regimen ↗

There are no guaranteed "safe days". Act on fever, sudden illness or a new serious symptom whenever it occurs. Do not wait for the next appointment.

Days 1, 8 and 15 (if prescribed)

Day 1 → Day 28: a general pattern

Day 22 is omitted only if the three-visit schedule is prescribed. Follow your confirmed appointments and prescription.

Treatment or check	Timing in this view
Paclitaxel Day 1	 **Day 1** IV
Paclitaxel Day 8	 **Day 8** IV
Paclitaxel Day 15	 **Day 15** IV
Nerves / muscle aches	 **Days 1–28** May persist or accumulate
Blood counts / energy	 **Days 1–28** Review before each dose; report illness any day

Weekly visits: Each planned infusion needs its own symptom and blood-test review.

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Between-treatment notes

Day 1: First visit of this cycle

Review symptoms, blood results and premedication before the infusion. Tell the nurse straight away about itching, flushing, throat or chest tightness, back pain, dizziness or breathing change. Check transport if premedication makes you drowsy and confirm the next weekly visit.

Day 2–7: Between weekly doses

Aches, fatigue, nausea or bowel symptoms may occur. Tingling may be mild at first but matters before the next dose. Record what symptoms stop you doing. Contact the team for new or worsening numbness, balance trouble, marked weakness or uncontrolled symptoms. Fever or sudden illness needs immediate contact.

Day 8: Second weekly review

This dose requires its own symptom and blood-test review. Describe the worst symptoms since the previous visit, not only how you feel today. Report sensory changes before the infusion and confirm your premedication instructions.

Day 9–14: Keep tracking function

Repeated treatment can increase nerve symptoms and fatigue. Check practical tasks such as buttons, writing and walking. Use the symptom passport to show changes. Seek early advice rather than waiting until numbness is severe.

Day 15: Third weekly review

The team decides whether treatment can proceed using the current assessment. Confirm whether Day 22 is planned for you; do not infer this from a different person's calendar.

Day 16–21: After the third visit

Symptoms may settle or continue. An interval between appointments does not rule out infection or a treatment complication. Use the urgent contact plan if unwell and bring your record to the next review.

Day 22: Day 22: follow the prescribed schedule

A fourth dose is part of the four-visit plan. In the consultant-selected three-visit plan there is no Day 22 infusion. The calendar here is educational. Use your confirmed appointment and prescription; call if the schedule is unclear.

Day 23–28: Complete the cycle record

Record symptoms and functional changes, including those that improve before the next appointment. Treatment continuation depends on benefit, toxicity and your priorities. Check the next cycle date and review plan. Do not treat a gap in visits as a guarantee of recovery.

Support around the medicines

1 Premedication

Premedication reduces infusion-reaction risk. The first-dose and later-dose plans may differ under the NCCP weekly pathway. The team must reconcile your exact product and reaction history; do not omit premedication yourself.

2 Nausea, bowel symptoms and pain

Use your individual anti-sickness and pain plan. Contact the team if symptoms are uncontrolled or you cannot drink. Antihistamines and other support medicines may cause drowsiness; arrange transport if advised.

3 Nerves and daily activities

Report tingling, numbness or weakness early, including effects on buttons, writing, walking or balance. Protect hands and feet and take care with hot surfaces if sensation changes.

4 Infection and blood counts

Each treatment needs its own blood and symptom review. A growth-factor injection may be prescribed after individual risk assessment; follow its separate instructions. Fever or sudden illness needs immediate contact on any day.

Practical support

Bring your medicine list and symptom record to every review.

Medicines and vaccines

Check new prescriptions, over-the-counter medicines, supplements and vaccine timing with the oncology pharmacist. Interactions and changes in organ function can alter the treatment plan.

Fertility and contraception

Discuss fertility preservation before treatment where relevant. Ask for written contraception and breastfeeding advice for your actual product and the period after treatment. Tell the team promptly about a possible pregnancy.

Support and priorities

Ask about symptom control, nutrition, rehabilitation, emotional support and help at home. Review how the treatment burden and possible benefit fit your priorities.

Questions worth asking

What benefit is realistic for me and what other options are available?

Treatment aims to control small-cell lung cancer after previous therapy and may help symptoms. The evidence is limited, so discuss likely benefit, burdens and alternatives with the team. A small uncontrolled study found some tumour responses to weekly paclitaxel in previously treated SCLC. The study used six weekly doses followed by two weeks without treatment, which differs from this NCCP schedule. It cannot establish a survival advantage over other treatment or supportive care.

Who arranges and reviews my blood tests and scans?

FBC, kidney and liver profile at baseline and before each treatment; NCCP also explicitly lists Day 8 FBC. Check neuropathy, daily function, infection and previous infusion reactions before every dose. Confirm whether Day 22 is prescribed. Ask the team who books each test and who checks the result.

What premedication and anti-sickness plan should I follow?

Follow the written plan for your exact prescription. Report missed premedication before infusion; confirm prevention and rescue medicines with the pharmacist.

Which symptoms need an immediate call, and which number should I use?

Call the oncology contact number immediately for fever, shivers or sudden illness, severe or new breathing difficulty, chest pain, fainting, severe abdominal pain or persistent diarrhoea. Tell staff immediately about infusion symptoms. Report new numbness, weakness, painful skin changes, swelling or visual changes promptly; do not wait for the next cycle. Keep the unit's daytime and out-of-hours numbers on your treatment card.

> **Try explaining the plan back:** Can you explain the treatment aim, your next visit, the blood-test plan, premedication and when to call for urgent help?

Paclitaxel

- Low blood counts / infection
- Sensory or motor neuropathy, potentially persistent
- Hypersensitivity / infusion reaction
- Hair loss and nail / skin changes
- Muscle and joint aches
- Fatigue and mouth / bowel symptoms
- Rare serious lung or cardiac effects
- Serious infection / sepsis
- Rare serious bowel inflammation; vision changes

Treatment scope · NCCP 00226 NCCP 00226d: small-cell lung cancer that has returned or has not responded to previous treatment. This use is outside the Irish licence and requires the hospital off-label approval and consent process. This guide is for conventional paclitaxel alone.

Supportive prescription draft for review. This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Paclitaxel · lung supportive care

Active medicines: Paclitaxel

Emetogenic risk: low

Medicine	Dose / route	When
Ondansetron	8 mg orally	On treatment day Review QT, constipation and hepatic dosing; reconcile any steroid already prescribed
Metoclopramide	10 mg orally up to three times daily as needed	Subsequent days: duration to be confirmed Assess contraindications, renal function and interactions

Additional phase-specific support

- NCCP first-dose suggested premedication: dexamethasone 8 mg intravenous (IV), chlorphenamine 10 mg IV and famotidine 20 mg IV 30 minutes before infusion.
- From Day 8, NCCP permits omission of routine dexamethasone; previous reaction or higher risk requires consultant-directed adjustment. Famotidine omission is also a consultant/local-policy decision with close observation.
- The actual conventional product SmPC uses a different generic premedication schedule. Reconcile the exact product and local protocol; no automatic patient-led omission or switch to nab-paclitaxel.
- Consider analgesia for muscle/joint aches; review neuropathy before every dose and growth-factor support when indicated.
- Low emetic risk: prevention and rescue medicines remain separate from reaction premedication. Confirm individual suitability, interactions and duration; this reference is not a prescription.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

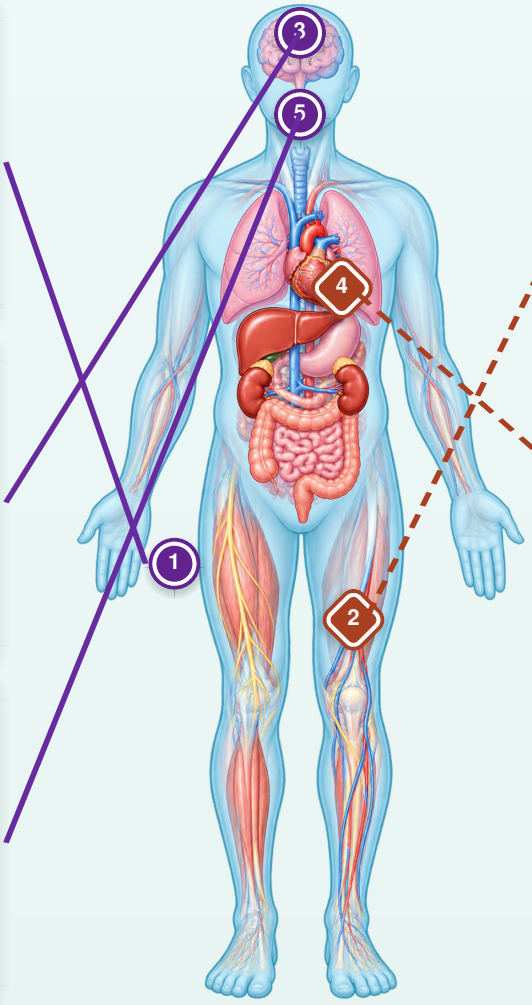
Weekly paclitaxel · relapsed or refractory small-cell lung cancer (SCLC)

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Source reconciled; independent clinical, pharmacy and patient/PPI review pending. Selected effects; not a complete medicine leaflet.

A picture of side effects

Numbered symptom boxes connect to the anatomical illustration. The medicine responsible may be uncertain; report the symptom.



1 Hands / feet

Paclitaxel

Report numbness, weakness or changes in walking and hand use before the next dose.

2 Blood / infection

Paclitaxel

Fever, shivers or sudden illness need immediate contact.

3 Hair / scalp

Paclitaxel

Hair loss can occur; ask about support before treatment.

4 Breathing

Paclitaxel

New or worsening breathlessness or cough needs urgent assessment.

5 Mouth

Paclitaxel

Report painful ulcers, difficulty drinking or poor intake.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

The body map highlights symptoms to report. It does not show where your cancer is or predict which effects will occur.

Call the oncology contact number immediately for fever, shivers or sudden illness, severe or new breathing difficulty, chest pain, fainting, severe abdominal pain or persistent diarrhoea. Tell staff immediately about infusion symptoms. Report new numbness, weakness, painful skin changes, swelling or visual changes promptly; do not wait for the next cycle.

CHAPTER 04

Sources and further information

IN THIS CHAPTER

The references behind this information and addresses you can use to find further help.

Use this information alongside the plan agreed with your treatment team. Your team confirms your medicines, doses and appointments.

1. **NCCP 00226 v9 · official regimen ↗**
   https://healthservice.hse.ie/documents/6562/226_V9_Paclitaxel_80.pdf
2. **HPRA conventional paclitaxel SmPC; 9 January 2026**
   https://assets.hpra.ie/products/Human/27560/Licence_PA2315-115-001_09012026104101.pdf
3. **Yamamoto et al., Anticancer Research 2006; relapsed / refractory SCLC**
   https://pubmed.ncbi.nlm.nih.gov/16739353/
4. **NCCP NCIS medical-oncology antiemetics V8, low risk**
   https://healthservice.hse.ie/documents/7180/Supportive_Care_Antiemetics_for_inclusion_NCIS_Medical_Oncology.pdf